M O N D E R M A N

The Quarter-Trillion Dollar Friction: Administrative Complexity in U.S. Healthcare

How administrative complexity absorbs a quarter-trillion dollars and diverts institutional capacity from patient care.

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Executive Summary

The United States healthcare system spent $5.3 trillion in 2024—18 percent of GDP and roughly twice the average of comparable high-income nations. Across independent sources, a consistent picture emerges: a substantial share of that spending is consumed by administrative complexity rather than patient care. This paper organizes that evidence around three findings.

1. Administrative overhead is the system's largest category of waste. A 2019 JAMA systematic review estimated total healthcare waste at $760–$935 billion annually, with administrative complexity accounting for $266 billion—the single largest category. A 2021 McKinsey/Harvard analysis identified over $250 billion in potential savings from administrative simplification alone.

2. The overhead absorbs clinician time and degrades care delivery. AMA survey data show physicians and staff spend approximately 14 hours per week on prior authorization alone. Physicians report spending roughly one-third of their work time on non-patient-facing administrative tasks. These diversions do not merely cost money; they slow treatment, contribute to burnout, and reduce the system's capacity to convert spending into health outcomes.

3. The problem is structural, not incidental. U.S. administrative costs run 3 to 8 times higher than peer nations. The gap is driven not by clinical complexity but by system design: fragmented payer interfaces, non-standardized billing, manual prior authorization, and over 1,700 quality measures. Reform efforts have produced incremental gains without altering the underlying architecture.

The paper concludes with four structural pathways: billing simplification, prior authorization reform, interoperability infrastructure, and quality measurement rationalization.

How This Paper Uses Evidence

Peer-reviewed medical literature (JAMA, Health Affairs, Annals of Internal Medicine) provides the most rigorous evidence on waste categories, administrative cost estimates, and cross-national comparisons. These are cited with full publication details.

Government data and analyses (CMS National Health Expenditure Accounts, CMS Office of the Actuary) provides authoritative spending figures. These are the official U.S. government estimates of national health expenditures.

Professional surveys and policy research (AMA, AAFP, Commonwealth Fund, OECD, McKinsey/Harvard) provides data on clinician time burden, cross-national benchmarks, and simplification opportunities.

Where the paper synthesizes across categories, it says so. Figures are labeled as direct data, derived comparison, or framework as appropriate.

1. The Overhead Is the System's Largest Category of Waste

What the system spends

U.S. healthcare spending reached $5.3 trillion in 2024, or $15,474 per person, according to the CMS National Health Expenditure Accounts published in Health Affairs. This represents 18 percent of GDP—up from 5 percent in 1960 and 13.3 percent in 2000. The U.S. spends approximately twice as much per capita on healthcare as comparable high-income nations, a gap that has been documented repeatedly by the Commonwealth Fund, the OECD, and in peer-reviewed research.

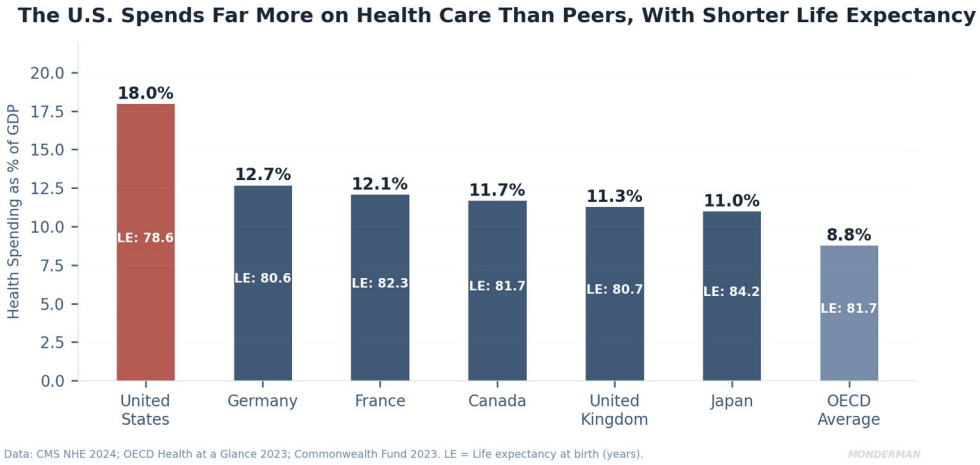

Figure 1. The U.S. spends far more on healthcare than peer nations, with shorter life expectancy. Data: CMS NHE 2024; OECD; Commonwealth Fund, 2023. LE = life expectancy at birth.

Where waste accumulates

A landmark 2019 JAMA systematic review by Shrank, Rogstad, and Parekh estimated total waste in the U.S. healthcare system at $760 billion to $935 billion annually—approximately 25 percent of total spending. The study examined six waste domains identified by the Institute of Medicine and Donald Berwick's foundational 2012 JAMA analysis. Administrative complexity was the largest single category at an estimated $265.6 billion. Pricing failure accounted for $231–$241 billion. Failure of care delivery, overtreatment, fraud, and care coordination failures comprised the remainder.

Notably, Shrank et al. found no published studies evaluating interventions to reduce administrative complexity—the largest waste category had attracted the least systematic reform effort. Berwick, in an accompanying JAMA editorial, observed that under current payment systems, many waste-reduction methods would reduce profit for the organizations that employ them. This structural misalignment helps explain why administrative overhead has remained roughly constant as a share of spending for over three decades.

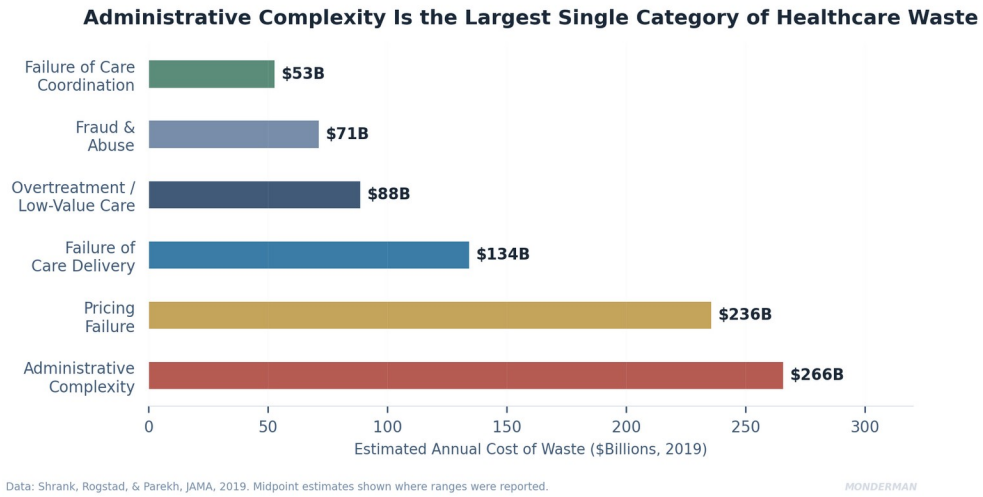

Figure 2. Administrative complexity is the largest single category of healthcare waste. Data: Shrank, Rogstad, & Parekh, JAMA, 2019. Midpoint estimates where ranges were reported.

2. The Overhead Absorbs Clinician Time and Degrades Care Delivery

The prior authorization burden

The American Medical Association's 2022 survey of 1,001 practicing physicians found that 88 percent reported the burden of prior authorization as "high or extremely high." Practices completed an average of 45 prior authorization requests per physician per week. Physicians and staff spent approximately 14 hours per week—nearly two full business days—on prior authorization tasks. One-third of practices employed staff who worked exclusively on prior authorization. Ninety-four percent reported that prior authorization delayed necessary care, and 33 percent reported that a patient had experienced a serious adverse event as a result.

A 2023 KFF analysis of CMS data found that Medicare Advantage insurers received more than 50 million prior authorization requests in 2023, partially or fully denying 3.2 million. Among denials that were appealed, 81.7 percent saw the initial denial overturned—suggesting that a substantial share of the administrative burden produces denials that do not survive scrutiny.

The broader time burden

AMA data for 2024 show that physicians reported a 57.8-hour average workweek, spending 27.2 hours on direct patient care, 13 hours on indirect patient care (documentation, order entry, test interpretation), and 7.3 hours on administrative tasks including prior authorization and insurance forms. The Medscape Physician Compensation Report for 2023 estimated physicians spend an average of 15.5 hours per week on paperwork and administration. A 2020 study found physicians spend approximately one-third of their total work time on non-patient-facing EHR, administrative, and other tasks.

These are not incidental costs. They represent a structural diversion of clinical capacity. The American Academy of Family Physicians has described prior authorization as the "bane of our existence" for practicing physicians, noting that practices have had to hire multiple additional full-time employees solely to manage authorization processes—even when their approval rate exceeds 99 percent.

3. The Problem Is Structural, Not Incidental

Cross-national evidence

Papanicolas, Woskie, and Jha, writing in JAMA (March 2018), compared healthcare spending across 11 high-income countries. They found that administrative costs accounted for 8 percent of total U.S. health expenditures, compared with a range of 1 to 3 percent in peer nations. The U.S. spent approximately twice as much as comparators overall, yet utilization rates were largely similar. The spending gap was driven primarily by prices for labor and goods, pharmaceuticals, and administrative costs—not by greater use of services.

Richman, Kaplan, and colleagues, in a 2022 Health Affairs cross-national study, found that billing and insurance-related costs for a single inpatient surgical bill ranged from $6 in Canada to $215 in the United States. Himmelstein and colleagues have documented that hospital administrative costs in the U.S. account for approximately 25 percent of total hospital expenditures—the highest among eight nations studied, and rising.

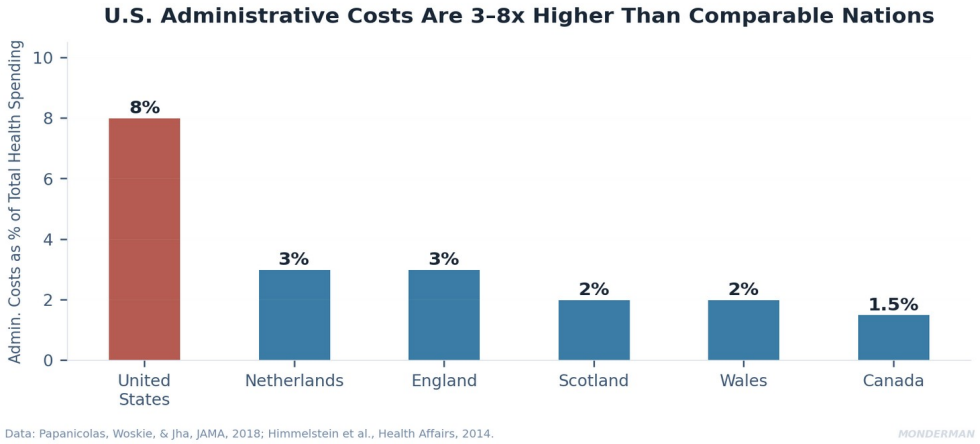

Figure 3. U.S. administrative costs are 3–8x higher than comparable nations. Data: Papanicolas et al., JAMA, 2018; Himmelstein et al., Health Affairs, 2014.

Why the architecture produces overhead

This paper terms the condition accumulated administrative drag: the compounding effect of fragmented payer interfaces, non-standardized processes, and layered compliance requirements that individually appear rational but collectively absorb institutional capacity far beyond what comparable systems require. Several structural drivers sustain this condition:

Payer fragmentation. The U.S. system requires providers to interact with hundreds of distinct insurance plans, each with different formularies, prior authorization requirements, claims formats, and appeals processes. McKinsey's 2021 analysis identified approximately $200 billion in annual spending on financial transactions alone—claims processing, payment, patient collections, and prior authorization.

Misaligned incentives. As Kocher noted in a 2021 JAMA Viewpoint, administrative complexity is profitable. Payers use prior authorization and claims processes to reduce medical costs; providers add staff to navigate those processes; and both sides

invest in systems that manage complexity rather than eliminate it. The result is an equilibrium that no single actor has incentive to disrupt.

Regulatory accumulation. CMS requires reporting on more than 1,700 quality measures. Physicians spend the time equivalent of seeing nine patients per week on quality reporting alone, according to research cited in the McKinsey analysis. Each measure may serve a legitimate purpose; collectively, they constitute a substantial reporting burden that has grown faster than any systematic review has rationalized it.

4. Pathways Forward: A Structural Framework

Sahni, Carrus, and Cutler (McKinsey/Harvard, 2021) estimated that administrative simplification could save over a quarter-trillion dollars annually—with $175 billion achievable without new legislation. Their framework categorized interventions as "within" (individual organizations), "between" (requiring inter-organizational agreement), and "seismic" (requiring industry-wide or legislative change). This paper adapts that taxonomy into four structural pathways.

Figure 4. Four structural pathways for reducing administrative drag in U.S. healthcare. Design framework. Sources: McKinsey/Harvard (Sahni et al., 2021); JAMA; AMA; CMS.

Billing and payment simplification addresses the fragmented payer interface consuming approximately $200 billion annually. First step: standardize claims formats across payers and automate routine adjudication. Measure: administrative cost per claim processed. Risk: payer resistance and regulatory complexity across state and federal jurisdictions.

Prior authorization reform targets the 14+ hours per week of clinician time diverted from care. First step: implement gold-card exemptions for providers with approval rates above 90 percent, as Texas has legislated. Measure: prior authorization volume per physician per week. Risk: utilization may increase without accompanying clinical safeguards; reform must balance access with appropriate stewardship.

Data and interoperability infrastructure addresses redundant data collection and siloed systems. First step: accelerate adoption of FHIR-based data exchange and move toward a universal patient identifier. Measure: percentage of financial and clinical transactions that are fully electronic. Risk: privacy concerns, implementation costs, and resistance from legacy system vendors.

Quality measurement rationalization addresses the 1,700+ measures creating reporting burden without proportionate outcome improvement. First step: reduce to a core set of outcome-based measures, retiring process measures that have not demonstrated meaningful clinical impact. Measure: physician hours spent on quality reporting per week. Risk: accountability gaps if measures are retired without replacement; reform requires CMS leadership and stakeholder consensus.

5. Conclusion

Across peer-reviewed research, government data, and professional surveys, the broad contours of the problem are consistent. The U.S. healthcare system carries administrative overhead that substantially exceeds international benchmarks—not because American medicine is more complex, but because the administrative architecture through which care is financed, authorized, documented, and measured has accumulated more friction than comparable systems. The cost is measured in dollars, clinician hours, and the institutional capacity that is diverted from patient care.

Administrative complexity is the largest single category of healthcare waste, estimated at $266 billion annually in the most rigorous available review. It absorbs roughly two full days of physician and staff time per week in prior authorization alone. And it has proven resistant to reform because the current architecture creates an equilibrium in which complexity is profitable for multiple parties, even as it degrades system performance.

The pathways forward are structural. Billing simplification, prior authorization reform, interoperability infrastructure, and quality measurement rationalization are not new ideas. What has been absent is the institutional willingness to treat them as design problems rather than policy debates. The U.S. healthcare system does not need to become simpler. It needs to become more navigable—a system whose administrative architecture converts complexity into care rather than drag.

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